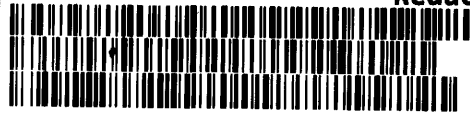


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TCGA-XF-A9T0-01A-PR

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Collected:
Ordered by:

DIAGNOSIS:

RADICAL CYSTOPROSTATECTOMY, LEFT NEPHROURETERECTOMY, LYMPH NODE DISSECTION AND CREATION OF STUDER NEO-BLADDER:

RIGHT DISTAL URETER (A):

INTRAOPERATIVE FROZEN SECTION DIAGNOSIS:

- NO HIGH GRADE ATYPIA OR TUMOR IDENTIFIED

FINAL DIAGNOSIS:

- BENIGN URETER
- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

APICAL URETHRAL MARGIN (B):

INTRAOPERATIVE FROZEN SECTION DIAGNOSIS:

- NO HIGH GRADE ATYPIA OR TUMOR IDENTIFIED

FINAL DIAGNOSIS:

- BENIGN URETHRAL AND PERIURETHRAL TISSUE
- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

BLADDER / PROSTATE (C):

BLADDER:

- INVASIVE POORLY DIFFERENTIATED UROTHELIAL CARCINOMA (7.6 X 3.3 X 2.2 CM), GRADE 4/4, GROSSLY AND MICROSCOPICALLY EXTENDING THROUGH WALL OF BLADDER NECK TO INVOLVE PERIVESICAL SOFT TISSUE
- MULTIFOCAL (INCLUDING TRIGONE AND LEFT URETEROVESICAL JUNCTION) UROTHELIAL CARCINOMA IN SITU (FLAT LESION)
- LEFT ATTACHED URETER, REACTIVE CHANGES WITHOUT UROTHELIAL DYSPLASIA OR MALIGNANCY
- RIGHT ATTACHED URETER, NO SIGNIFICANT HISTOPATHOLOGIC CHANGE
- RESECTION MARGINS, FREE OF UROTHELIAL DYSPLASIA AND MALIGNANCY

PROSTATE:

- UNILATERAL PROSTATIC ADENOCARCINOMA, GLEASON'S SCORE 7 (3+4), CONFINED TO PROSTATE WITHOUT CAPSULE OR PERIPROSTATIC SOFT TISSUE INVOLVEMENT
- MULTIFOCAL HIGH GRADE PROSTATIC INTRAEPITHELIAL NEOPLASIA (PIN III)
- PROSTATIC URETHRA, NO UROTHELIAL DYSPLASIA OR UROTHELIAL MALIGNANCY IDENTIFIED
- BILATERAL SEMINAL VESICLES, FREE OF MALIGNANCY
- RESECTION MARGINS, FREE OF MALIGNANCY AND UROTHELIAL DYSPLASIA

LEFT KIDNEY (D):

- STENOSIS OF LEFT URETERAL PELVIC JUNCTION WITH ASSOCIATED MILD PELVIC DILATION
- URETER, URETERAL PELVIC JUNCTION AND RENAL PELVIS EXHIBITING NO UROTHELIAL DYSPLASIA OR MALIGNANCY
- RENAL PARENCHYMA, NO SIGNIFICANT HISTOPATHOLOGIC CHANGE
- NO MALIGNANCY IDENTIFIED IN SIX HILAR LYMPH NODES EXAMINED (0/6)

RIGHT PARACAVAL LYMPH NODES (E):

- NO MALIGNANCY IDENTIFIED IN FIVE LYMPH NODES EXAMINED (0/5)

RIGHT COMMON ILIAC LYMPH NODES (F):

- NO MALIGNANCY IDENTIFIED IN FIVE LYMPH NODES EXAMINED (0/5)

LEFT PARA AORTIC LYMPH NODES (G):

- NO MALIGNANCY IDENTIFIED IN EIGHT LYMPH NODES EXAMINED (0/8)

LEFT COMMON ILIAC LYMPH NODES (H):

- NO MALIGNANCY IDENTIFIED IN FOUR LYMPH NODES EXAMINED (0/4)

ICD O-3
Carcinoma, urothelial NOS
812013
Site: Bladder NOS
C67.9
J23/26/14

Collected

Ordered by

Location

LEFT PARA AORTIC PROXIMAL LYMPH NODES (I):

- NO MALIGNANCY IDENTIFIED IN SIX LYMPH NODES EXAMINED (0/6)

RIGHT LYMPH NODES OF CLOQUET (J):

- NO MALIGNANCY IDENTIFIED IN THREE LYMPH NODES EXAMINED (0/3)

RIGHT EXTERNAL ILIAC LYMPH NODES (K):

- NO MALIGNANCY IDENTIFIED IN 14 LYMPH NODES EXAMINED (0/14)

RIGHT OBTURATOR / HYPOGASTRIC LYMPH NODES (L):

- NO MALIGNANCY IDENTIFIED IN SIX LYMPH NODES EXAMINED (0/6)

LEFT OBTURATOR / HYPOGASTRIC LYMPH NODES (M):

- NO MALIGNANCY IDENTIFIED IN FIVE LYMPH NODES EXAMINED (0/5)

LEFT EXTERNAL ILIAC LYMPH NODES (N):

- NO MALIGNANCY IDENTIFIED IN THREE LYMPH NODES EXAMINED (0/3)

RIGHT PRESACRAL LYMPH NODES (O):

- NO MALIGNANCY IDENTIFIED IN FOUR LYMPH NODES EXAMINED (0/4)

PRESACRAL LYMPH NODES (P):

- NO MALIGNANCY IDENTIFIED IN EIGHT LYMPH NODES EXAMINED (0/8)

LEFT PRESACRAL LYMPH NODES (Q):

- NO MALIGNANCY IDENTIFIED IN TWO LYMPH NODES EXAMINED (0/2)

RIGHT PROXIMAL URETER (R):

- BENIGN URETER
- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

LYMPH NODE SUMMARY: NO MALIGNANCY IDENTIFIED IN TOTAL OF 79
LYMPH NODES EXAMINED (0/79)

PATHOLOGIC TNM STAGE: BLADDER: pT1sT3bN0MX
PROSTATE: pT2aN0MX (AJCC-1997)

COMMENT:

Representative sections of invasive urothelial carcinoma are submitted for p53 assay by immunohistology, results of which will be reported separately.

SPECIMEN SOURCE:

- A: "Right distal ureter"
- B: "Apical urethral margin F/S"
- C: "Bladder/prostate"
- D: "Left kidney"
- E: "Right para caval lymph nodes"
- F: "Right common ilia lymph nodes"
- G: "Left para aortic lymph nodes"
- H: "Left common iliac lymph nodes"
- I: "Left para aortic proximal lymph nodes"

Collected
Ordered by

J: "Right lymph nodes of cloquet"
K: "Right external iliac lymph nodes"
L: "Right obturator/hypogastric lymph nodes"
M: "Left obturator/hypogastric lymph nodes"
N: "Left external iliac lymph nodes"
O: "Right pre-sciatic lymph nodes"
P: "Pre sacral lymph nodes"
Q: "Left pre-sciatic lymph nodes"
R: "Right proximal ureter"

CLINICAL INFORMATION:

Pre-Op Dx: bladder cancer; S/P hiatal hernia repair, TURP

GROSS EXAMINATION:

A: The specimen is received fresh from the O.R. and labeled "Right distal ureter". It consists of a portion of ureter with attached fat which measures 0.4 cm in diameter by 0.6 cm long. Entirely embedded for frozen section in cassette AFS.

B: The specimen is received fresh from the O.R. and labeled "Apical urethral margin F/S". It consists of two strips of soft pink mucosal tissue which together measures 1.5 x 0.5 x 0.4 cm. Entirely submitted for frozen section diagnosis in cassette BFS.

C: The specimen is received fresh from the O.R. and labeled "Bladder/prostate". It consists of a cystoprostatectomy specimen which includes bladder and prostate. Overall, the specimen measures 27.5 x 11 x 4.5 cm with a smooth, glistening, mobile peritoneum which measures 23.5 x 10 by less than 0.1 cm. The peritoneum does not appear to be involved by tumor. The bladder itself measures 7.4 x 5 x 3.5 cm. The specimen is palpated to reveal firmness involving the anterior bladder neck and this area is inked black on the outside as well as the entire prostate. The specimen is then opened along the anterior prostatic urethra to reveal a large, exophytic, fungating, firm, pink tumor involving most of the anterior bladder neck wall measuring 7.6 x 3.3 x 2.2 cm with the involved wall measuring 2 cm in maximal thickness. The tumor approaches within less than 0.2 cm of the inked resection margin. The tumor is split into half as the result of the opening incision. The uninvolved bladder wall measures 1.2 cm in thickness and the rest of the bladder mucosa appears unremarkable. Both ureterovesical junctions are identified and are patent. The attached ureters are opened longitudinally to reveal an unremarkable mucosa. The right ureter measures 6 cm in length and 0.5 cm in circumference, and the left ureter attached to kidney measures 20 cm in length and 0.6 cm in circumference. The prostate measures 5 x 4.5 x 4.5 cm. The seminal vesicles are palpated from the posterior side with the right seminal vesicle measuring 2 x 1 x 1 cm and the left seminal vesicle measuring 2.1 x 0.7 x 0.7 cm. The prostatic urethra appears excavated and measures 3.1 cm in length by 2.6 cm in circumference. There is a focal area of hemorrhage in the posterior wall of the urethra which measures 0.4 x 0.3 cm and which represents a possible biopsy site. The verumontanum itself measures 0.6 x 0.5 x 0.5 cm. Representative sections of the bladder tumor and bladder are submitted in cassettes C1 to C28. The prostate is then serially sectioned from apex to base (1 through 9) to reveal gross hypertrophy of the transitional zone. Sections are submitted in cassettes C29 to C51.

D: The specimen is received fresh from the O.R. and labeled "Left kidney". It consists of a nephroureterectomy specimen which is attached to the bladder and prostate by its ureter (bladder and prostate represent part C) and overall measures 21 x 12.4 x 4 cm, including the perinephric fat. The kidney itself measures 12 x 5 x 3 cm. The ureter attached to the bladder measures 20 cm in length and 0.6 cm in external diameter. The kidney is bivalved and reveals unremarkable cortex and renal medullary tissue. The renal pelvis appears slightly dilated. The junction of the proximal ureter and renal pelvis is narrowed to the extent that it is difficult to open utilizing scissors. Once opened the circumference at the

Temporary Label	
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Location	

narrowest point of the ureteropelvic junction is 0.4, but is otherwise unremarkable. The mucosa of the entire ureter, ureteropelvic junction and renal pelvis appears grossly normal. No mass lesions or other mucosal abnormalities are appreciated. The opened specimen is demonstrated at this point to the surgeon. The vascular margins including renal artery and vein are then identified with the renal artery measuring 3 cm in length by 0.5 cm in diameter and renal vein measuring 3 cm in length by 1 cm in diameter. The renal cortex measures 1 cm in maximal thickness and the medullary tissue 1 cm in thickness. The perirenal fat is serially sectioned but no adrenal gland is identified as well as no obvious hilar nodes. Representative sections are submitted in 11 cassettes.

E: The specimen is received in formalin and labeled "Right para caval Lymph nodes". It consists of a piece of soft yellow fatty tissue measuring 4 x 3 x 1 cm. No obvious lymph nodes are identified. Entirely submitted in one cassette.

F: The specimen is received in formalin and labeled "Right common iliac lymph nodes". It consists of a piece of soft yellow fatty tissue measuring 3 x 1 x 1 cm. No obvious lymph nodes are identified. Entirely submitted in one cassette.

G: The specimen is received in formalin and labeled "Left para aortic lymph nodes". It consists of a piece of soft yellow fatty tissue measuring 2.8 x 1.9 x 0.9 cm. No obvious lymph nodes are identified. Entirely submitted in one cassette.

H: The specimen is received in formalin and labeled "Left common iliac lymph nodes". It consists of two pieces of soft yellow fatty tissue measuring in aggregate 3.6 x 2.1 x 1.3 cm. No obvious lymph nodes are identified. Entirely submitted in one cassette.

I: The specimen is received in formalin and labeled "Left para aortic proximal lymph nodes". It consists of a piece of soft yellow fatty tissue measuring 4 x 1 x 1 cm. One possible lymph node is identified measuring 2.5 cm in greatest dimension. Entirely submitted in two cassettes.

J: The specimen is received in formalin and labeled "Right lymph nodes of cloquet". It consists of two pieces of soft yellow fatty tissue measuring in aggregate 2.6 x 2 x 0.6 cm. No obvious lymph nodes are identified. Entirely submitted in one cassette.

K: The specimen is received in formalin and labeled "Right external iliac lymph nodes". It consists of two large pieces of soft yellow fatty tissue measuring in aggregate 6 x 3.2 x 1.4 cm. No obvious lymph nodes are identified. Entirely submitted in three cassettes.

L: The specimen is received in formalin and labeled "Right obturator/hypogastric lymph nodes". It consists of a large piece of soft yellow fatty tissue measuring 4.5 x 3.7 x 1.4 cm. No obvious lymph nodes are identified. Entirely submitted in one cassette.

M: The specimen is received in formalin and labeled "Left obturator/hypogastric lymph nodes". It consists of a large piece of soft yellow fatty tissue measuring 6.4 x 4.1 x 1.4 cm. No obvious lymph nodes are identified. Entirely submitted in two cassettes.

N: The specimen is received in formalin and labeled "Left external iliac lymph nodes". It consists of a piece of soft yellow fatty tissue measuring 4.3 x 2.6 x 2.1 cm. One possible lymph node is identified measuring 2.6 cm in greatest dimension. Entirely submitted in four cassettes.

O: The specimen is received in formalin and labeled "Right pre-sciatic lymph

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Ordered by:

100-3101

nodes". It consists of multiple small pieces of soft yellow fatty tissue measuring in aggregate 2.7 x 1.9 x 0.6 cm. No obvious lymph nodes are identified. Entirely submitted in one cassette.

P: The specimen is received in formalin and labeled "Pre sacral lymph nodes". It consists of a large piece of soft yellow fatty tissue measuring 5.5 x 4 x 1.3 cm. No obvious lymph nodes are identified. Entirely submitted in three cassettes.

Q: The specimen is received in formalin and labeled "Left pre-sciatic lymph nodes". It consists of multiple small pieces of soft yellow fatty tissue measuring in aggregate 2.1 x 1.6 x 0.5 cm. No obvious lymph nodes are identified. Entirely submitted in one cassette.

R: The specimen is received in formalin and labeled "Right proximal ureter". It consists of a 0.6 cm in length and 0.4 cm in diameter portion of ureter with attached fat which has been stapled equidistant from both ends. Both ends are inked black. The specimen is serially sectioned and entirely submitted in one cassette.

SECTIONS:

AFS: frozen section, right distal ureter
 BFS: frozen section, apical urethral margin
 C1,2: bladder/prostate; anterior bladder wall
 C3: right ureterovesical junction
 C4: right lateral bladder wall
 C5: dome
 C6: posterior bladder wall
 C7: left lateral bladder wall
 C8: left ureterovesical junction representing first 4 cm of the attached distal left ureter with the more proximal section inked black
 C9: left ureter additional sections representing the last proximal 8 to 13 cm of the left ureter
 C10: trigone
 C11-14: tumor on the right side of bladder neck
 C15-26: tumor on the left side of bladder neck
 C27: right perivesical fat for possible lymph nodes
 C28: left perivesical fat for possible lymph nodes
 C29: distalmost prostate on the right side
 C30: distalmost prostate on the left side
 C31,32: right midprostate (slice #4), anterior and posterior
 C33,34: left midprostate (slice #4), anterior and posterior
 C35,36: right side of proximal prostate (slice #7), anterior
 C37,38: right side of proximal prostate (slice #7), posterior
 C39,40: left side of proximal prostate (slice #7), anterior
 C41,42: left side of proximal prostate (slice #7), posterior
 C43,44: left side of proximal prostate (slice #7), additional sections, posterior
 C45,46: right side of additional proximal prostate, posterior
 C47,48: left side of additional proximal prostate on the posterior side only
 C49,50: left side of proximalmost prostate adjacent to tumor
 C51: right side of proximalmost prostate adjacent to tumor
 D1: left kidney; renal artery and vein margins
 D2-5: ureteropelvic junction - entirely submitted
 D6: proximal ureter
 D7,8: renal pelvis, including the superior, mid and lower poles
 D9: renal parenchyma
 D10,11: hilar fat for possible lymph nodes
 E: right paracaval lymph nodes - all embedded

Ordered by

Location

F: right common iliac lymph nodes - all embedded
 G: left para-aortic lymph nodes - all embedded
 H: left common iliac lymph nodes - all embedded
 I1: left para-aortic proximal lymph nodes; large lymph node, bisected
 I2: remaining tissue
 J: right lymph nodes of Cloquet - all embedded
 K1-3: right external iliac lymph nodes - all embedded
 L: right obturator/hypogastric lymph nodes - all embedded
 M1,2: left obturator/hypogastric lymph nodes - all embedded
 N1,2: left external iliac lymph nodes; large lymph node, bisected
 N3,4: remaining tissue
 O: right presciatic lymph nodes - all embedded
 P1-3: presacral lymph nodes - all embedded
 Q: left presciatic lymph nodes - all embedded
 R: right proximal ureter - all embedded

INTRAOPERATIVE FROZEN CONSULTATION:

AFS: Right distal ureter:

- no high grade atypia or tumor identified

BFS: Apical urethral margin:

- no high grade atypia or tumor identified

MICROSCOPIC EXAMINATION:

A-B: See final microscopic-diagnosis.

C: Sections of the bladder show multiple foci of urothelial carcinoma in situ (flat lesion) [C2,3-5,7,8,10,11,13,14,16,18,20,21,24,26]. This includes the trigone (C10) and left ureterovesical junction (C8). Sections of the mass around the bladder neck show a deeply invasive poorly differentiated urothelial carcinoma (C11-26), that focally involves the perivesical soft tissue (C19,25,26). Sections of the attached left ureter show focal reactive changes but no evidence of dysplasia or malignancy. No lymphovascular invasion is seen and all resection margins are free of both urothelial dysplasia and malignancy.

Sections of the prostate show unilateral prostatic adenocarcinoma (C30,43), Gleason's score 7 (3+4). The carcinoma is confined to the prostate without involvement of either the capsule or periprostatic soft tissue. Also seen are several foci of high grade prostatic intraepithelial neoplasia (PIN III) [C29,34]. The prostatic urethra shows no evidence of urothelial dysplasia and no invasive urothelial carcinoma is seen in any of the sections of the prostate. Bilateral seminal vesicles show no evidence of malignancy. Resection margins are free of malignancy.

D: Sections of the kidney that include the entire ureteropelvic junction, show no urothelial dysplasia or malignancy. The mucosa of the renal pelvis is unremarkable. Sections of renal parenchyma show no significant histopathologic change. Six renal hilar lymph nodes are identified, all of which are free of malignancy and show no significant histopathologic features.

E-R: See final microscopic-diagnosis.

Criteria	Yes	No
Diagnosis Discrepancy		✓
Primary Tumor Site Discrepancy		✓
HIPAA Discrepancy		✓
Prior Malignancy History		✓
Dual/Synchronous Primary Noted		✓
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials: <i>W</i>	Date Reviewed: 1/11/14	